

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 76 yrs | Sex: Male | MRN: SMC-04471932
Encounter Date: February 28, 2022 | Provider: Brian Holt, MD — Gastroenterology | Doc ID: SMC-2022-0228-GI

Gastroenterology — Endoscopy Report

Colonoscopy & EGD for Iron-Deficiency Anemia

Indication

77-year-old with new microcytic anemia (Hgb 10.6 g/dL, ferritin low) found on routine labs; also due for colorectal screening. Anticoagulation (apixaban) held per protocol (last dose 48 h prior; bridging not required).

Pre-Procedure Labs

Test	Result	Units	Reference Range	Flag
Hemoglobin	10.6	g/dL	13.5 - 17.5	L
MCV	78	fL	80 - 100	L
Ferritin	18	ng/mL	30 - 400	L
Transferrin saturation	9	%	20 - 50	L
Creatinine	1.8	mg/dL	0.7 - 1.3	H

Colonoscopy Findings

Cecum reached; prep adequate. Moderate sigmoid diverticulosis without active bleeding. Two sessile polyps removed by snare/cold technique: 8 mm sigmoid and 6 mm descending colon (sent to pathology). No mass, no actively bleeding lesion. Internal hemorrhoids, non-bleeding.

Esophagogastroduodenoscopy (EGD) Findings

Mild antral gastritis; no ulcer or active bleeding. No varices. Duodenum normal; biopsies for celiac/H. pylori obtained. Findings may reflect chronic mucosal blood loss; gastritis possibly related to prior antiplatelet/anticoagulant exposure.

Assessment & Plan

Iron-deficiency anemia likely multifactorial: occult GI blood loss (diverticulosis/polyps, gastritis) plus anemia of CKD. No malignancy identified.

- Start oral ferrous sulfate 325 mg every other day (better tolerated/absorbed); recheck CBC/ferritin in 8 weeks; consider IV iron if intolerant or CKD-related.
- Start a proton-pump inhibitor (pantoprazole 40 mg daily) for gastritis.
- Resume apixaban 24 h post-polypectomy (no high-risk bleeding stigmata).
- Await polyp pathology (see report 03/05/2022). Repeat colonoscopy interval per pathology.

Medications Started

Medication	Dose	Route	Frequency	Indication
Ferrous sulfate	325 mg	PO	Every other day	Iron-deficiency anemia
Pantoprazole	40 mg	PO	Once daily	Gastritis / GI protection